

Physical disability education pedagogy in graduate and continuing medical education: a scoping review

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Introduction

Patients with disabilities constitute the largest underrepresented minority population in the USA, and yet they consistently encounter health disparities, as recently recognized by the NIH.

- Lack of physician education remains a major contributing factor.
- While organizational calls have led to improvements in undergraduate curricula, little has addressed the training needs of practicing physicians in **graduate** and **continuing** education

Objective

To address this disparity, we must assess existing efforts. Herein we explore the current literature for educational directives, curriculum, and studies concerned with the state of disability pedagogy in GME and CME.

Methods

- A multifactorial search query was conducted in PubMed, CINAHL, and Embase with a carefully crafted search string
- Inclusion criteria:
 - Emphasized physical disability education
 - Focused on practicing physicians *or* residents
 - No publication year, language, or location restrictions
- Manual abstract reviews and citation tracking were conducted to expand the search and include empirical studies, systematic reviews, and academic commentaries

Results

Seventeen articles met inclusion criteria, including quantitative surveys (*n*=6), qualitative interviews (*n*=4), commentaries (*n*=3), mixed-methods studies (*n*=1), and specific frameworks or guidelines (*n*=3). The study populations ranged from specific residents (pediatrics, emergency medicine), to fully trained physicians undergoing CME (*n*=10), to broader health-care providers (MPHs, clinical staff), to MedEd leaders (*n*=3). Recurring themes to inform future interventions centered on person-centered care (*n*=8), cultural competence (*n*=9), legal risk (*n*=3), trainee inclusion (*n*=3), and functional assessment (*n*=3).

Conclusion & Implications

This work showcases the variegated methods medical educators have employed to improve disability education in GME and CME. Specific themes can guide us toward high-yield interventions, including the development of functional assessments of disabled patients and legal fluency in providing care for different disabled populations. Above all, cultural competence and person-centered solutions should be prioritized via active inclusion of disabled patients, educators, and trainees as we continue to develop curricula.

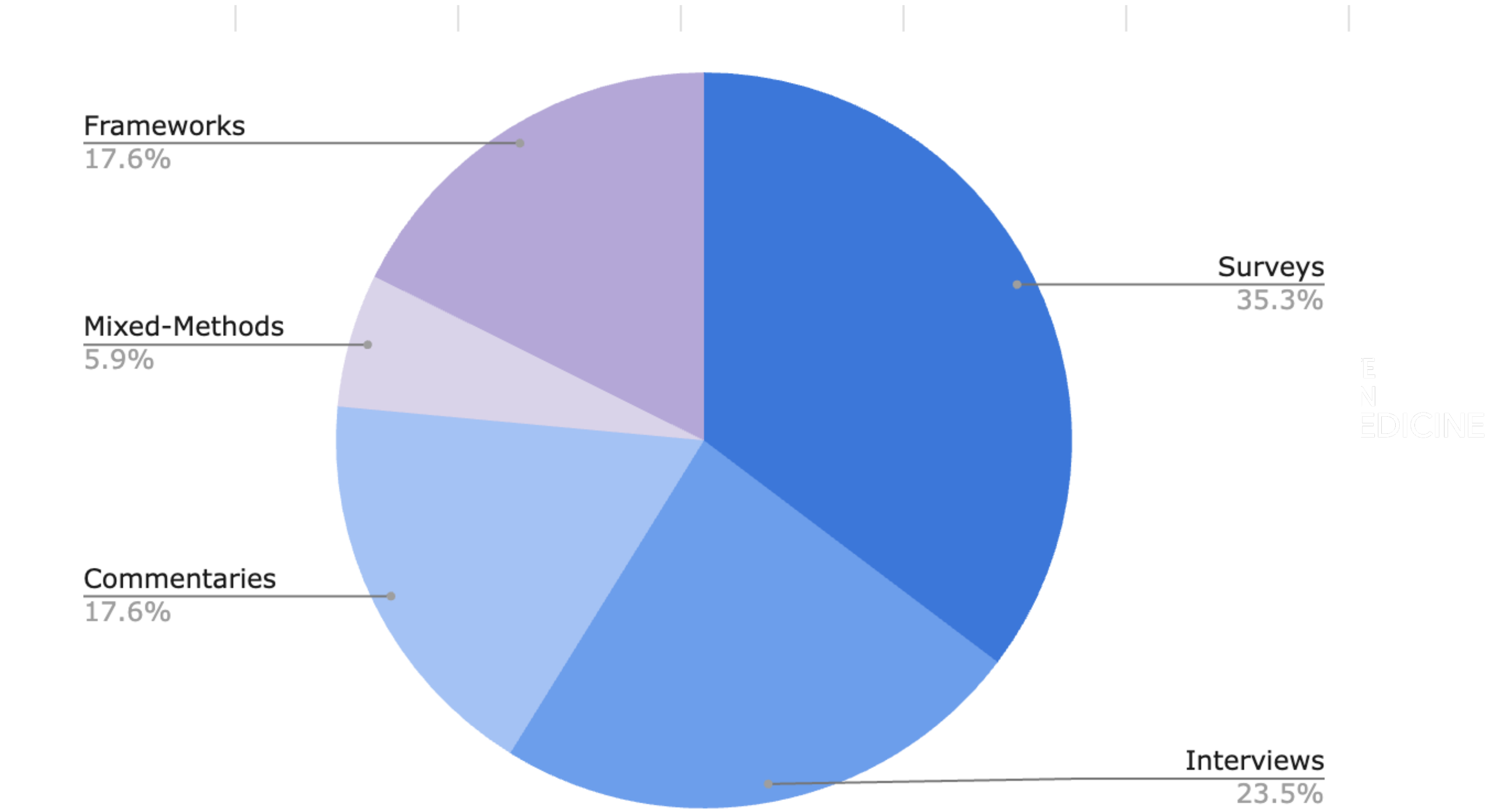


Figure 1. Types of studies identified in scoping review

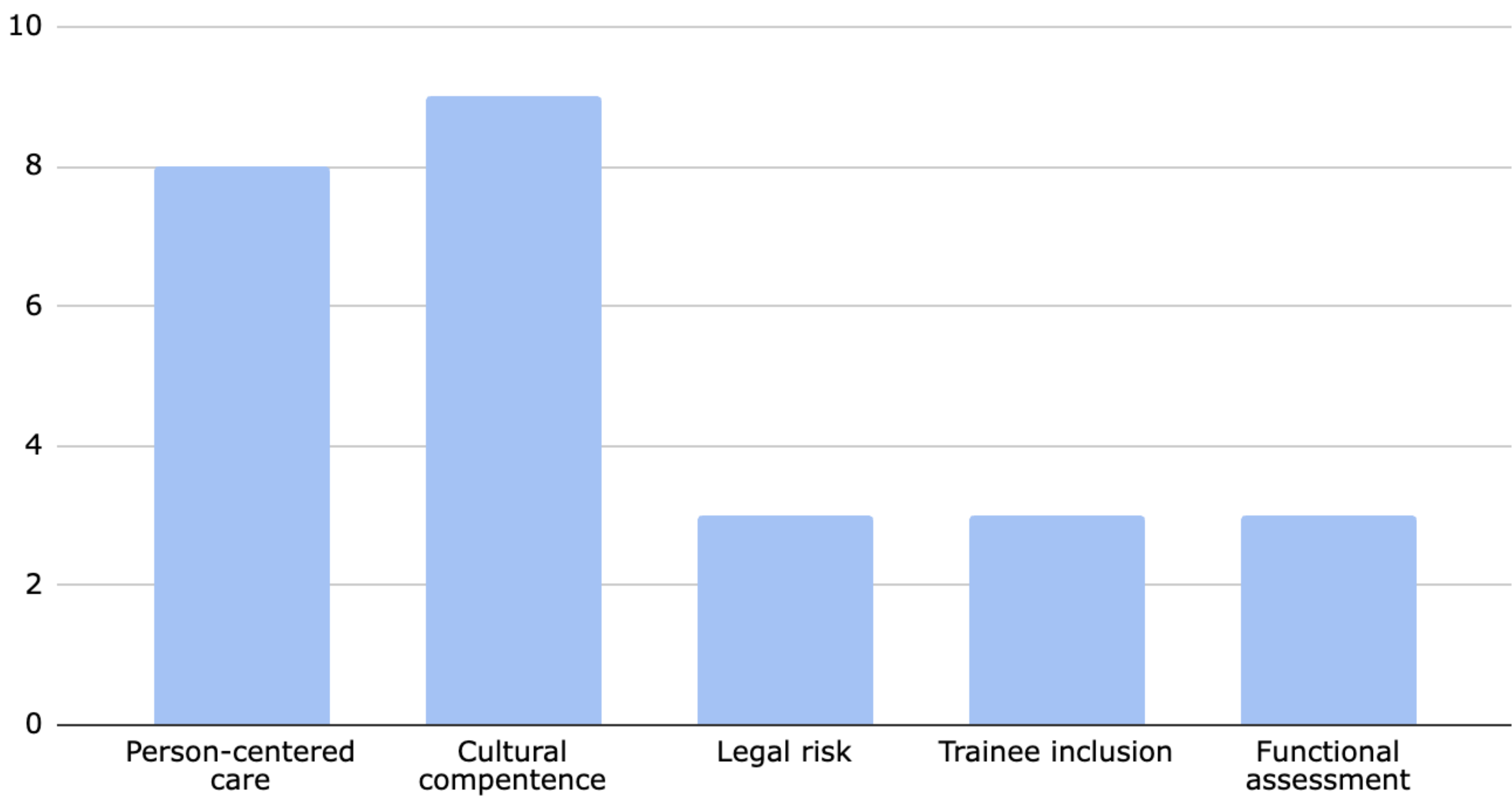


Figure 2. Types of interventions identified in reviewed articles

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